ETHICS FOR NURSES

What do we mean by Ethics?

- Tober (1965) defined ethics as "a system of moral principles governing the conduct السلوك of a nurse, his/her relationship with the patient, family and society."

- Cape B (1974) defined ethics as "a code of moral principles governing the nurse's behavior with his/her patient, friends and colleagues."

Cont. What do we mean by Ethics?

- Ethics originated from a Greek word "Ethos" meaning custom, habit, conduct.
- Ethics is defined as the science that approaches human actions in terms of right and wrong.
- It basically deals with duties and responsibilities as well as rights.

Cont. What do we mean by Ethics?

- **Ethics**, also known as moral philosophy الفلسفة الأدبية, is a branch of philosophy that **involves** systematizing, defending, and recommending concepts of right and wrong conduct.

- Ethics is a complement to علم الجماليات Aesthetics in the philosophy field of علم الفضائل Axiology.



Types of Ethics

Ethics may be divided into four major areas of study:

- Meta-ethics, "theoretical meaning and reference of moral propositions"

Question: What does 'right' even mean?

- Normative ethics, "practical means of determining a moral course of action"

Question: How should people act?

Cont. Types of Ethics

- Applied ethics "how moral outcomes can be achieved in specific situations.

Question: How do we take moral knowledge and put it into practice?

- Descriptive ethics, "also known as comparative ethics, is the study of people's beliefs about morality.

Question: What do people think is right?

The difference between Ethics and Morals

- The word "Morals" and "Ethics" originally meant much same thing:

- Ethics came from "**Greek**" while Morals came from "**Latin**" Both words refer to the general area of right and wrong in the theory and practice of human behavior.

- Morals refer to standards of behavior actually held or followed by individuals and groups, while Ethics refer to the science or study of morals and values, it's an activity in the academic context.

Comparison between morals and ethics

Morals	Ethics
• **Principles and rules of right conduct** • **Private and personal** • **Commitment to principles and values is usually defended in daily life** • **Pertain to an individual's character**	• Formal responding process used to determine right conduct • Professionally and publicly stated • Inquiry or study of principles and values • Process of questioning and perhaps changing one's morals • Speaks to relationship between human being

MORALS

- Standards of right and wrong.
- Learned by socialization (society & culture play important role).
- Based on religious believes.
- Learned & internalized at early age.

Moral distress

- When the health care professionals are unable to follow their moral beliefs because of institutional or other restriction.
- The distress occurs when the health care professional violates a personal moral value and fails to fulfill perceived responsibility.
- Moral distress represent practical, rather than ethical dilemmas.

Moral outrage

- Occurs when someone else in the health care setting performs an act the health care professional believes to be immoral.
- Health care professional do not participate in the act.
- Health care professional not responsible for wrong but perceives that he is powerless to prevent.

Ethical Terms

- **Bioethics:**
  - Interdisciplinary field within health care that has evolved with modern medicine to address questions that arise as science and technology produce new ways of knowing.
  - Physicians, nurses, social workers, psychiatrists, philosophers, and theologians are joining to address ethical questions in health care.

Ethical Terms



- Ethical Code: الدستور الاخلاقي A written list of professions values and standards of conduct.
- Statement of etiquette: بيان للآداب describe expected professional behaviors established in ethical codes.
- Laws: rules of social conduct معايير designed to prevent the actions of infringing يخرق on the right of others.

Cont. Ethical Terms

- Ethical dilemma: المعضلة الاخلاقية situation requires a choice between two equally unfavorable alternatives.

- Accountability: المسئولية answerability or responsibility.
  - Personal: to one self and patient.
  - Public: to employer and society.

Cont. Ethical Terms

- Attitudes: المواقف manner, way, position and feeling toward a person.
- Believes: المعتقدات building blocks of attitude, the strength of attitudes depend on how strongly believes are held to the extent to which they are consistent to each other.
- Attitudes are changeable and easy to change.
- Values are difficult to change.



Cont. Ethical Terms

- **Rights:** entitlements الاستحقاقات that one deserves according to just claims ادعاء صادق, legal guarantee, or moral principles.

Rights are claims that you deserve something from someone or some group.





Cont. Ethical Terms

Rights are also categorized as positive and negative. This categorization indicates the kind of claim and obligation that the right entails.

- Positive Right - If I have a positive "Right to X," then that means that society has an obligation to provide me with X. This is also called an entitlement right.
- For example; if the right to health care is a positive right of all citizens, then the society must provide health care to all citizens.

Cont. Ethical Terms

- Negative Right - If I have a negative "Right to X," then that means that society has an obligation to prevent undue interference from my obtaining and keeping X if I choose to do so.
- For example; if the right to health care is a negative right of all citizens, then the society must prevent undue interference with citizens' access to and use of health care services.

Cont. Ethical Terms

Common good

- Best for community/society.
- Decisions should be made on what is good for the community as a whole, not necessarily for the individual. Where many of our nations laws are base.

Utilitarian

- Most good, least harm.

Cont. Ethical Terms

- Moral Agent - an individual who consciously acts and can therefore be held responsible for his/her actions. For example; newborns infants are not considered moral agents because they lack the capacity for agency.

Cont. Ethical Terms

- Motive - A motive is what caused the agent to choose this action.
  For example; if a person was moved by compassion to act, the motive was compassion. Motives may be conscious or unconscious.

Cont. Ethical Terms

- Intention - An intention is the desired purpose or aim of the agent's action. Intentions are conscious. People disagree about the degree to which intentions matter in determining whether an action is right or wrong.

Cont. Ethical Terms

- For example; imagine two people, Ann and Jan. Each hits their husband with the car, killing him. Ann intended to kill her husband. Jan intended only to back out of the driveway, she hit her husband by accident. Most would agree that Ann's action is morally worse than Jan's because of the intention.

Values and values clarifications

Values

- **Values**: are freely chosen, enduring beliefs or attitudes about the worth of a person, object, idea, or action (e.g. freedom, family, honesty, hard work).
- **Value:** personal belief about worth that acts as a guide to behavior.
- **Value system:** entire framework on which actions are based.

Values and values clarifications

Three-steps of valuing

1. Choosing: beliefs are chosen, from among alternative.
2. Prizing: the beliefs that are selected are cherished.
3. Acting: the selected beliefs are demonstrated consistently through behavior.

Theory of Values Clarification (Raths, Harmin, and Simon, 1978)

Valuing Process		
Choosing	Cognitive	•Beliefs are freely chosen. •Without outside pressure •After reflecting and considering consequences or alternatives
Prizing	Affective	•Chosen beliefs are cherished or prized
Acting	Behavior	•Chosen beliefs are affirmed to others, incorporated into one's behavior and repeated consistently in one's life.

Cont. Values and values clarifications

- Although health care professionals cannot and should not ignore or deny their own and the profession's values, they need to be able to accept a client's values and beliefs rather than assume their own are the "right ones"
- So, health care professionals often need to behave in a value-natural way (i.e. being nonjudgmental).

Ethical Principles (Concepts)



Purpose of ethical principles

- Establish common ground among nurse, patient, family, other health care professionals, and society for discussion of ethical questions and ethical decision making.
- Permit people to take a consistent position on specific or related issues.
- Provide an analytical framework by which moral problems can be evaluated.

Ethical Principles (Concepts)



- **Beneficence**-الفائدة، الاحسان compassion; taking a positive action to help others; desire to do good; core principle of our patient advocacy.
- obligation to do good and not harm(do only good).
- Example: An elderly patient falls at home and has a fractured hip. In the emergency room, we act to provide pain medication as soon as possible is an act of beneficence.
- "vaccination, health education"

Cont. Ethical Principles



- **Nonmaleficence** - منع الاذى avoidance of harm or hurt; core of medical ethics.
- Often in modern times, nonmaleficence extends to making sure you are doing no harm in the beneficent act of using technology to extend life or in using experimental treatments that have not been well tested.
- Example: "Prevent cross infection"

Cont. Ethical Principles



- **Autonomy**— الاستقلالية agreement to respect another's right to self-determine تقرير المصير a course of action; support of independent decision making.
- Example: In clinical situations health professionals respect a patient's autonomy, where the patient is allowed the freedom of choice regarding treatment, such as in deciding whether he/she wishes to be intubated during an exacerbation of COPD, or deciding when he/she wishes to forgo امتنع further dialysis. If a patient lacks capacity for such a decision and has an advance directive, the person who has the durable power of وكيل attorney can make the decision "consent form".

Cont. Ethical Principles



- **Justice-**العدالة Derived from the work of John Rawls, this principle refers to an equal and fair distribution of resources, based on analysis of benefits and burdens of decision.
- Justice implies that all citizens have an equal right to the goods distributed, regardless of what they have contributed or who they are. For example; we all have rights to services from the postal service, firefighters, police, and access to public schools, safe water, and sanitation.
- "provide care to all regardless of ability of pay".

Cont. Ethical Principles



- **Fidelity-** الموالاةThis principle requires loyalty, truthfulness مصداقية, advocacy and dedication to our patients.
- It involves an agreement to keep our promises. Fidelity refers to the concept of keeping a commitment and is based upon the فضيلة virtue of caring.



Cont. Ethical Principles



- Example: A patient asks the health professional not to reveal the fact that she is dying or give her diagnosis to her family. The health professional asks why she does not want her family advised. The patient explains that her family is very emotional and has stated they would do everything to keep her alive, even if it required long-term mechanical ventilation. The patient has explained multiple times that she does not want mechanical ventilation. The health professional recognizes that keeping of this information in confidence, while supporting the family.

Cont. Ethical Principles



- **Veracity-** قول الحقيقة Obligation to tell the truth.

Example: " don't lie, don't give false results."

- **Confidentiality-**الثقة Feeling with trust.

Example: "Don't tell anyone about patient's chart or secrets."

- **Respect-**الاحترام For patient autonomy, dignity and liberty.

Example: "Respect patient customs and religion during treatment."

Cont. Ethical Principles



- **Paternalism-** حماية الضعفاء Health care professionals make decisions about diagnosis, therapy and prognosis for the patient.
- Protect the weak people in the community.
- "Protect the children, handicapped, geriatrics and being present during emergencies".
- Based upon the health care professional's belief about what is in the best interest of the patient, he/she chooses to reveal or withhold patient information in these three important arenas. This principle is heavily laden حمل as an application of power over the patient.

Cont. Ethical Principles



- **Altruism-** الايثارProtect the community and meet the health needs.

Example:" concern for welfare of others."

- **Egoism-** المنفعة الشخصية لتطوير الذاتThe right to maximize personal benefits.

Example: "develop your self by upgrading or taking degrees."

- **Utilitarianism-** منفعة لأكبر عدد ممكن The greatest good for the greatest number.

Example: " give a lecture to a big number and make courses for a big number. "

Ethical Theories

- Ethical theories attempt to provide a system of principles and rules for resolving ethical dilemmas.
- Ethical theories consist of fundamental beliefs about what is morally right or wrong and propose reasons for maintaining these beliefs.
- Ethical theories provide the bases for professional codes of ethics.

Classes of Ethical Theories

There are many classes of ethical theories that are used most often for moral reasoning in professional practice.

- **TELEOLOGICAL** الغائية
- **CONSEQUENTIALIST** العواقب
- **DEONTOLOGICAL** الواجب
- **RELATIVISM/SUBJECTIVISM** النسبية
- **VIRTUE BASED THEORIES** الطهارة



Teleological Theory

- The term Teleological Ethics describes a group of theories that are focused on the outcomes or consequences of actions, and so this group of theories are also described as Consequentialist Ethics.
- The word Teleology derives from the Greek word *telos*, meaning end or goal.
- It is also known as utilitarianism because the ultimate ethical test for any decision is based on what is the best for most of people.

Cont. Teleological Theory

- Theory which judges the rightness of an action in terms of an external goal or purpose.
- consequences always play some part in the determination of what one should or should not do.
- Thus, the choice that benefits many people justifies harm that may come to a few.

Cont. Teleological Theory

- Stated from a different perspective, teleologists believe "the end justifies the mean".
- A teleologist would argue that selective abortion (destroying some embryos in a multiple pregnancy) is ethical because it is done to ensure the full-term birth of those that remain.
- In other words, termination can be justified in some situations but may not be justified in all cases.

Cont. Teleological Theory

- **نظرية المنفعة أو الفائدة**: حيث يحكم على الفعل بأنه جيد أو سيء من خلال مقارنته بنتائجه جيدة أم سيئة؟؟ وبأن المنفعة تتحقق بتقديم أقصى ما يمكن من إيجابيات وبأقل ما يمكن من سلبيات، إلا أنها أخفقت حين تعاملت مع الأفعال الغير أخلاقية على أساس المنفعة.

Cont. Teleological Theory

Benefits

- There is room in some theories for good intentions, even if the action didn't active the desired end.
- Active attempt to connect morality with the "real" world.
- By allowing for the consideration of consequences, teleological theories can adapt to different circumstances and situations.

Consequentialist Theory

- Consequences of an action determine its moral value.
- Consequences can be good, neutral, or evil.
- It based on Utilitarianism:
  - Rooted in the assumption that an action or practice is right if it leads to the greatest possible balance of good consequences, or to the fewest possible bad consequences.

Cont. Consequentialist Theory

Benefits

- Consequentialism is grounded in actual effect. So, moral action always improves life on earth.
- Often attentive to the particulars of the situation.
- Allow for exceptions to the rule when warranted مكفول by the outcome.
- Can be proven wrong or right by referring to empirical evidence, instead of a theoretical ideal.

Cont. Consequentialist Theory

Problems

- Consequences are difficult to predict.
- "Does the end always justify the means?"
- A consequentialist theory would justify many actions that we normally would consider wrong, if it turned out that the consequences were good.

Cont. Consequentialist Theory

Cont. Problems

- Undermines (weaken) trust in others and intimate (Close) relationships since we can never be sure that the consequences might not justify a betrayal خيانة (cheating) of trust and in many of these theories, each individual is treated the same regardless of one's relationship.

Deontological Theory

- **Deontology** derives from the Greek words for duty *(deon)* and science (or study) of *(logos)* علم الواجبات
- Is ethical decision making based on duty or moral obligations.
- A decision is right only if it conforms to an overriding moral duty and wrong only if it violates that moral duty
- All decisions are made in such a way that the decision could become universal law.
- Claims that there are features within the actions themselves which determine whether or not they are right.

Deontological Theory

- It proposes that the outcome is not the primary issue; rather, decisions must be based on the morality of the act itself.
- Do not consider consequences to be important.
- In other words, certain actions are always right or wrong regardless of circumstances.
- Deontologists would argue that destroying any fetus is wrong, whether done to save others or not, because killing is immoral.

Cont. Deontological Theory

- Immanuel Kant's ethical theory is deontological.
- He claims that actions are only morally right when they are done out of duty.
- He sees moral duties as unchanging laws for human conduct.
- He believes that morality is derived from the ability to think rationally, which enables beings to be free.

Cont. Deontological Theory

Benefits

- Right and wrong actions are easily determined by considering one's duties.
- Unlike utilitarianism, the end does not justify the means. Deontological theories provide a sound basis for inalienable (Unchangeable) rights and inherent value.
- Since duties do not change, there is a greater sense of security/predictability in the accepted behavior of others.
- Good motives are valued, even if the outcome wasn't what you expected.

Cont. Deontological Theory

Problems

- There is no agreement on a single standard for morality.
- Ignoring consequences can cause pain and suffering.
- The imposition of a specific moral belief system on others has been a cause of significant harm throughout history.

Virtue Based Theories

- Virtue based theories focus on the character of the person. According to virtue based theories, ethics is about what sort of person one should strive to become. The qualities that one should develop in oneself are called virtues (e.g. honesty, fairness, kindness, faithfulness, generosity, prudence, integrity, bravery, etc). One should act in ways that develop these virtuous qualities within oneself.

Cont. Virtue Based Theories

- ***For example,*** Aristotle claimed that "in order to become an honest person, one should tell the truth". Eventually it becomes a habit. Along, the way one learns how to tell the truth appropriately, without being brutally honest all of the time or lying whenever it is easier to do so.
- There are many virtues that one ought to develop through practice over one's lifetime.
- Becoming virtuous is excelling at all of the virtues that make a good human being, health care professional, etc.
- It is a learning process that continues throughout your life.

Cont. Virtue Based Theories

Benefits

- This type of theory recognizes that individuals and circumstances are unique. For example, the virtue of compassion may be expressed by two people in two different ways. Similarly, running into a burning building may be courageous action for a fire professional but foolhardy for an untrained individual with no protective equipment.
- Virtue ethics allows each individual to use his/her own judgment when making difficult moral decisions, yet recognizes certain common goals.
- Mistakes are expected and recognized as learning opportunities.

Cont. Virtue Based Theories

Problems

- Some argue that too much is left to individual judgment, thus opening the door to bias and prejudice.
- Similarly, virtues can be interpreted very differently. For example, consider the many ways that fairness may be interpreted.
- Virtue ethics depends on modeling for some of the education. However, one may choose a poor role-model and therefore develop a false sense of virtue.

Relational or "Caring" Ethical Theories

- These theories bring an entirely new perspective to the process of moral reasoning for ethical issues in health care. Actions are not judged according to the outcome or the principles of duty and obligation. But rather a perspective of caring and responsibility is used to determine what might be a morally correct act.
- In others words the morally good act is the one that shows caring and concern for other people and what might be important to them.

Code of Ethics for Nurses

Code of Ethics

Definition of Ethical Codes:

- Ethical codes are adopted by organizations to assist members in understanding the difference between 'right' and 'wrong' and in applying that understanding to their decisions.

Nursing Codes of Ethics

- Purposes:
  - **Inform the public** about the minimum standards of the profession and help them understand professional nursing conduct.
  - Provide **a sign of** the profession's **commitment** to the public it serves.
  - **Outline the major ethical considerations** of the profession.
  - **Provide standards** for professional behaviors.
  - **Remind nurses of special responsibility** they assume when caring for the sick.

Code of Ethics

Health Professionals have four fundamental responsibilities:

- To promote health.
- To prevent illness.
- To restore health.
- To alleviate suffering.

Code of Ethics

Health care is respectful and unrestricted by considerations of:

Age, colour, creed, culture, disability or illness, gender, sexual orientation, nationality, politics, race or social status.

Code of Ethics

ELEMENTS OF THE CODE

1. Nurses and people
2. Nurses and practice
3. Nurses and the profession
4. Nurses and co-workers

Code of Ethics - ELEMENTS OF THE CODE

1. Nurses and people

- The nurse's primary professional responsibility is to people requiring nursing care.
- In providing care, the nurse promotes an environment in which the human rights, values, customs and spiritual beliefs of the individual, family and community are respected.

Code of Ethics - ELEMENTS OF THE CODE

1. Nurses and people

- The nurse ensures that the individual receives accurate, sufficient and timely information in a culturally appropriate manner on which to base consent for care and related treatment.

- The nurse holds in confidence personal information and uses judgment in sharing this information.

Code of Ethics - ELEMENTS OF THE CODE

1. Nurses and people

- The nurse shares with society the responsibility for initiating and supporting action to meet the health and social needs of the public.
- The nurse advocates for equity and social justice in resource allocation, access to health care and other social and economic services.
- The nurse demonstrates professional values such as respectfulness, responsiveness, compassion, trustworthiness and integrity.

Code of Ethics - ELEMENTS OF THE CODE

2. Nurses and practice

- The nurse carries personal responsibility and accountability for nursing practice, and for maintaining competence by continual learning.

- The nurse maintains a standard of personal health such that the ability to provide care is not compromised.

Code of Ethics - ELEMENTS OF THE CODE

2. Nurses and practice

- The nurse uses judgment regarding individual competence when accepting and delegating responsibility.

- The nurse at all times maintains standards of personal conduct which reflect well on the profession and enhance its image and public confidence.

Code of Ethics - ELEMENTS OF THE CODE

2. Nurses and practice

- The nurse, in providing care, ensures that use of technology and scientific advances are compatible with the safety, dignity and rights of people.

- The nurse strives to foster and maintain a practice culture promoting ethical behavior and open dialogue.

Code of Ethics - ELEMENTS OF THE CODE

3. Nurses and the profession

- The nurse assumes the major role in determining and implementing acceptable standards of clinical nursing practice, management, research and education.

- The nurse is active in developing a core of research-based professional knowledge that supports evidence-based practice.

Code of Ethics - ELEMENTS OF THE CODE

3. Nurses and the profession

- The nurse is active in developing and sustaining a core of professional values.

- The nurse, acting through the professional organization, participates in creating a positive practice environment and maintaining safe, equitable social and economic working conditions in nursing.

Code of Ethics - ELEMENTS OF THE CODE

3. Nurses and the profession

- The nurse practices to sustain and protect the natural environment and is aware of its consequences on health.

- The nurse contributes to an ethical organizational environment and challenges unethical practices and settings.

Code of Ethics - ELEMENTS OF THE CODE

4. Nurses and co-workers

- The nurse sustains a collaborative and respectful relationship with co-workers in nursing and other fields.
- The nurse takes appropriate action to safeguard individuals, families and communities when their health is endangered by a co-worker or any other person.
- The nurse takes appropriate action to support and guide co-workers to advance ethical conduct.

Palestine council of health Code of professional conduct

Purpose:

- This code aims at maintaining professional behavior among nursing personnel, thus contributing to protecting the rights of clients to quality services. This code aims at reinforcing nurse's self-concept and self-esteem.

Palestine council of health Code of professional conduct

Definition of terms:

- **Conduct**: refers to the behavior of all categories of nursing personnel. Behavior includes your actions when on and off duty.
- **Nursing personnel categories**: are as defined by the national plan for the development of nursing in Palestine.
- **Accountability**: responsibility about all over your work.
- **The profession**: refers to nursing with all its specialties.
- **Professional knowledge**: refers to the nursing knowledge acquired from formal and informal educational opportunities that guide competence in practice.
- **Client**: refers to all consumers of health care services.

Palestine council of health Code of professional conduct

I. As a member of the nursing profession, you are personally accountable for your conduct **TOWARDS YOURSELF** therefore, you must:

- Use your professional knowledge to ensure safe nursing practices .
- Use your knowledge in health promotion and protection.
- Be responsible for improving the quality of nursing care by keeping up with professional development and continuously upgrading your knowledge.

Palestine council of health Code of professional conduct

I. As a member of the nursing profession, you are personally accountable for your conduct **TOWARDS YOURSELF** therefore, you must:

- Recognize limitations within your competence and your knowledge and decline duties which you are unable to perform as a result of these limitations. "This includes competence in using equipment and technology".
- Accept responsibility for the outcome of your decisions and actions.
- Participate in the development and implementation of health policies.

Palestine council of health Code of professional conduct

I. As a member of the nursing profession, you are personally accountable for your conduct **TOWARDS YOURSELF** therefore, you must:

- Ensure that you are physically fit to carry out your duties.
- Ensure that you are not under the influence of drugs or alcohol. Also, do not exhaust yourself by maintaining two full time jobs.
- Abide by the institution's regulations with regards to your professional attire.

Palestine council of health Code of professional conduct

I. As a member of the nursing profession, you are personally accountable for your conduct **TOWARDS YOURSELF** therefore, you must:

- Avoid wearing jewelry and slippers, knitting, chewing gum or eating bizzer when on duty. Take off the uniform when off duty and wear identity badges when on duty.
- Refrain from smoking when on duty and abide by hospital polices in regards to this matter.
- Avoid accepting gifts from advertising agents.

Palestine council of health Code of professional conduct

II. As a member of the nursing profession, you are personally accountable for your conduct TOWARDS YOUR CLIENTS. There for, you must:

- Recognize the individual needs of clients and their families. "This is reflected by your care plans that address the physical, social, psychological, intellectual, cultural and spiritual needs and values of clients".

Palestine council of health Code of professional conduct

II. As a member of the nursing profession, you are personally accountable for your conduct TOWARDS YOUR CLIENTS. There for, you must:

- Respect the individuality of clients and their right in maintaining their dignity.
- Avoid loud voice. Call clients by name and maintain their privacy and, avoid violence with clients including restraining patients for your comfort.
- Treat all clients with respect regardless of age, gender, class, religion or race.
- Protect and encourage client's independence and self care.

Palestine council of health Code of professional conduct

II. As a member of the nursing profession, you are personally accountable for your conduct TOWARDS YOUR CLIENTS. There for, you must:

- Report to a person in charge any circumstance, behavior or setting that could affect the physical, social or psychological welfare of clients and hinder you from delivering quality care.

Palestine council of health Code of professional conduct

II. As a member of the nursing profession, you are personally accountable for your conduct TOWARDS YOUR CLIENTS. There for, you must:

- Maintain the confidentiality of client's information and use discretion in sharing that information. "Maintain confidentiality when you access computers. Use judgment in sharing information and comply with the law when disclosing information to police. And comply with institutional policy in giving information to the media making sure that you always protect the client's right to privacy".

Palestine council of health Code of professional conduct

II. As a member of the nursing profession, you are personally accountable for your conduct TOWARDS YOUR CLIENTS. There for, you must:

- Document all treatments given to clients.
- Treat all documents as official records and refrain from altering them. "Avoid using correction fluid (tipex) and erasers".

Palestine council of health Code of professional conduct

II. As a member of the nursing profession, you are personally accountable for your conduct TOWARDS YOUR CLIENTS. There for, you must:

- Be aware of the ethical implications of research carried out on clients and ensure consent. "Provide appropriate information to enable informed judgment to be made by clients".
- Maintain a professional relationship with all clients. "This includes not accepting personal gifts or favors ".

Palestine council of health Code of professional conduct

III. As a member of the nursing profession, you are personally accountable for your conduct **TWOARDS OTHER PROFESSIONALS**.

Therefore you must:

- Work in a cooperative and collaborative manner with other health professionals and respect their contribution in the delivery of care.
- Report to an authorized person where the health or safety of your colleagues is at risk.

Palestine council of health Code of professional conduct

III. As a member of the nursing profession, you are personally accountable for your conduct **TWOARDS OTHER PROFESSIONALS**.

Therefore you must:

- Transmit knowledge, skills and attitudes to staff and students in your charge.
- Be responsible for the overall care provided by staff and students in your charge.
- Recognize the limitations of staff and students when delegating tasks.

Palestine council of health Code of professional conduct

III. As a member of the nursing profession, you are personally accountable for your conduct **TWOARDS OTHER PROFESSIONALS**.

Therefore you must:

- Report to an authorized person any incident which might occur from staff and students in your charge.
- Contribute actively to managing conflicts and solving problems.
- "Control your emotions and accept constructive criticism".

Palestine council of health Code of professional conduct

IV. As member of the nursing profession you are personally accountable for your conduct **TWOARDS THE PROFISSION** therefore you must:

- Contribute to the expanding body of nursing theory and its applications. "You can contribute within your abilities and circumstances".
- Be aware of the ethical implications of policies and / or procedures in your area of practice.

Palestine council of health Code of professional conduct

IV. As member of the nursing profession you are personally accountable for your conduct **TWOARDS THE PROFISSION** therefore you must:

- Develop and share nursing knowledge through research.
- Refrain from participating in any form of torture of imprisonment.
- Ensure that people trust and respect your profession. "Behaviors when off duty are a reflection of your profession".
- Maintain and support the professional credibility of nurses.
- Maintain the property of clients and workplace.

Palestine council of health Code of professional conduct

IV. As member of the nursing profession you are personally accountable for your conduct **TWOARDS THE PROFISSION** therefore you must:

- Support other nurses in their professional roles.
- Be responsible for furthering the goals of the profession. "Support and participate in activities that lead to the development of the profession".
- Support the philosophy and polices of your institution. "Be aware of the philosophy and polices of your institution and participate in developing them towards professionalism".

Palestine council of health Code of professional conduct

V. As member of the nursing profession you are personally accountable for your conduct **TWOARDS THE ENVIRONMENT** therefore you must:

- Show interest in the society's problems that affect health.
- Realize that your personal health attitude influence your community and surroundings.
- Take into account the economical aspects in nursing.

Palestine council of health Code of professional conduct

V. As member of the nursing profession you are personally accountable for your conduct **TWOARDS THE ENVIRONMENT** therefore you must:

- Cooperate with other professions in promoting community and national efforts to meet the health needs of the public.
- Participate in health campaigns and educational activities organized by the community. And cooperate with other professions in maintaining a healthy environment and / or controlling health hazards.

Medical Ethics in Islam

- The medical profession was a well-respected specialty and its leaders kept it this way by laying down proper ethics. Al-Tabari, the chief health professional in 970 A.D., described the Islamic code of ethics as follows (Hamamch 1971, Levy 1967).

Cont. Medical Ethics in Islam

I. Personal characters of the health professional:

- The health professional ought to be modest, virtuous, merciful, and unaddicted to liquor.
- He should wear clean clothes, select his company to be persons of good reputation.
- He should be careful of what he says and should not hesitate to ask forgiveness if he has made an error.
- He should be forgiving and never seek revenge.
- He should be friendly and peacemaker.
- He should not make jokes or laugh at the improper time or place.

Cont. Medical Ethics in Islam

II. His obligation towards patients:

- He should avoid predicting whether a patient will live or die, only God (Allah) knows.

- He ought not loose his temper when his patient keeps asking questions, but should answer gently and compassionately.

- He should treat alike the rich and the poor. Beliefs that God will reward him if he helps the needy.

- The health professional should not be late for his rounds or his house calls. He should be punctual and reliable.

- No matter how much he is paid. He should not give drugs to a pregnant woman for an abortion unless necessary for the mother's health.

Cont. Medical Ethics in Islam

III. His obligation towards the community:

- The health professional should speak no evil of reputable men of the community or be critical of any one's religious belief.

Cont. Medical Ethics in Islam

IV. His obligations towards his colleagues:

- Speak well with his colleagues.
- He should not honor himself by shaming others.
- If another health professional has been called to treat his patient, the family doctor should not criticize his colleague.
- He must warn him that combining different types of therapy may be dangerous because the actions of different drugs may be incompatible and injurious.

Cont. Medical Ethics in Islam

V. His obligations towards his assistants:

- If his subordinate does wrong, the health professional should not rebuke him in front of others, but privately and cordially.

Patient's Rights and Responsibilities

- We have to teach the patient that being a good patient does not mean being a silent one.

- That we have to encourage patients to provide us about his/her questions, problems, safety concerns, or unmet needs.

Patient's Rights

- Right to respectful care.
- Right to request the name of his attending physician.
- Right to consideration of his privacy.
- Right to have all medical records to be confidential.
- know what hospital rules and regulations.
- Right to expect emergency procedures without unnecessary delay.
- Right to good quality care and high professional standards.

Cont. Patient's Rights

- Right to full information concerning his diagnosis, treatment, and prognosis.
- Right to obtain the necessary informed consent prior to the start of any procedure or treatment, or both.
- Right to refuse drugs, treatment, or procedure offered by the hospital.
- Right to assistance in obtaining consultation with another physician.

Cont. Patient's Rights

- Right to medical and nursing services without discrimination based upon race, color, religion, sex, national origin or source of payment.
- Right and need for effective communication.
- Right to have his or her cultural, psychosocial, spiritual and personal values, beliefs and preferences respected.

Patient Responsibilities

A Patient has the responsibility to...

- Provide information about present and past illnesses, hospitalizations, medications and other matters relating to your health history.
- Have members of your family authorized to review your treatment, if you are unable to communicate with doctors or nurses.
- Formulate an advanced directive and appoint a surrogate to make healthcare decisions on your behalf, to the extent permitted by the law.

Cont. Patient Responsibilities

A Patient has the responsibility to...

- Ask questions if you do not understand directions or procedures.
- Help your doctor, nurse, and healthcare support staff in their efforts to care for you by following their instructions and medical orders.
- Report safety concerns immediately to your doctor, nurse, or any healthcare support staff.
- Ask for pain relief when pain first begins and tell your doctor or nurse if your pain is not relieved.
- Avoid drugs, alcoholic beverages or toxic substances, which have not been administered by your doctor.

Cont. Patient Responsibilities

A Patient has the responsibility to...

- Accept medical consequences if you do not follow the care, service, or treatment plan provided to you.
- Help control noise and the number of visitors in your room (or exam room, if you are an outpatient).
- Respect the property of other people.
- Use the call light provided for your safety.
- Be considerate of other patients.

Cont. Patient Responsibilities

A Patient has the responsibility to...

- Sign a written acknowledgement that you have received the applicable Notice of Privacy Practices.
- Provide accurate information needed for processing your insurance coverage.
- Be responsible for payment of all services.

Nurses' Rights and Responsibilities

Nurses' Rights

i. Nurses have the right to be recognized as health professionals with a defined scope of practice that is unique to the profession.

ii. Nurses have the right to be recognized as practitioners of quality, cost-effective healthcare. They need to be compensated in a manner equal with their responsibilities and with the quality of care they provide. Those responsibilities and that compensation must be supported by public policy.

Nurses' Rights

iii. Nurses have a right to a seat at the table where healthcare policy is formulated so that they can speak to the issues involved in establishing and maintaining healthcare delivery systems.

iv. Nurses have the right to lead and direct their own practice.

v. Nurses have the right to personnel and material resources commensurate with the area and scope of their responsibilities.

Nurses' Responsibilities

i. Nurses have the responsibility to advance their professional abilities through ongoing professional education.

ii. Nurses have the responsibility to collaborate with one another and with other health care provider colleagues to develop and implement integrated models for healthcare delivery through research, education, and practice. These collaborations must also work at developing public policy to improve the health of the populace for whom they care.

Nurses' Responsibilities

iii. Nurses have the responsibility to advocate for advances in healthcare policy and legislation.

iv. Nurses have the responsibility to promote nursing's image, and knowledge of nursing's role in the healthcare delivery system to each other, to other professionals and to the general public.

Nurses' Responsibilities

v. Nurses have the responsibility to be advocates for and promoters of health for all persons.

Nurses are responsible to work in collaboration with other healthcare providers, with community groups, and with the legislatures to increase the years of healthy life of the people in the community and to eliminate disparities in healthcare delivery systems.

LEGAL CONCEPTS AND RESPONSIBILITIES

Definition of Law القانون is that it concerns rules and regulations by which a society is governed.

Distinction between Law and Ethics

Concepts	Law	Ethics
Source	External	Internal
Concerns	Conduct and Actions	Motive, attitude, culture
Interests	Society	Individual
Enforcements	Courts	Ethics Committee and professional organizations

Cont. Law

Sources of Law

- The most basic source of law is constitutional law القانون الدستوري
- Statutory laws القوانين التشريعية

  Are those rules and regulations enacted by the legislative branch of the government.

Cont. Law

Sources of Law

- Administrative laws القوانين التنفيذية

  Are enacted through the decisions and rules of administrative agencies, which are specific governing bodies charged with implementing selected legislation.

Cont. Law

Sources of Law

- Judicial laws القوانين القضائية

  Are created by courts of law, interpreting legal issues that are in dispute متنازع عليها

Divisions or Types of Law

I- Common and Civil Law

- Common law is a system of law that is derived from judges' decisions (which arise from the judicial branch of the government), rather than statutes or constitutions (which are derived from the legislative branch of government).

Cont. Divisions or Types of Law

- The terms common law and civil law often are used interchangeably to describe law that affects individuals rather than state entities.
- However, the main difference is that common law draws abstract rules from specific cases, whereas civil law starts with abstract rules, which judges must then apply to the various cases before them.

Cont. Divisions or Types of Law

- Common and civil law are divided into a variety of legal specializations, including:

1- Tort law

2- Contract law.

Cont. Divisions or Types of Law

1- Tort law "قانون الأضرار "

- H.C. Providers are more likely to become involved in civil litigation ادعاء مدني rather than criminal litigation ادعاء جنائي
- A tort is a wrongful act committed مرتكب against another person or the person's property متاع شخص
- These wrongful acts result in an injury or harm.

Categories of Torts

A- Intentional Torts: الجنح المقصودةActs in which the outcome was planned although the person may not have expected the outcome to harm the other person.

- **Types of intentional torts are:**

- **Assault:** To threaten or attempt to touch a person without consent. Example: Patient's orders call for IM injection; patient refuses to be given the injection & H.C.P. tells patient she is going to give the shot anyway.
- **Battery:** Touching without consent (Treating without consent).
- **False Imprisonment:** Unwarranted use of restraints or restrictions, restraining a person in AMA situations.

Cont. Categories of Torts

B- Quasi-intentional torts

Injury to economics and dignity

- **Types of quasi-intentional torts are:**

- **Invasion of Privacy:** Breach of confidentiality, photographing patients without their consent.
- **Defamation of Character:** التشهير Slander (verbally) or libel (written).

Examples: Talking about a patient to another health care provider in the elevator with other people present; disclosing patient information over the telephone; telling stories about a fellow coworker.

Cont. Categories of Torts

C- Unintentional torts

Acts in which the outcome was not intended to happen.

- **Types of unintentional torts are:**
  - **Negligence:** الاهمال The failure to act as a reasonably prudent person would have acted in a specific situation. (Applies to ALL people, even unlicensed caregivers).
  - **Malpractice:** سوء التصرف The failure of a professional to use such care as a reasonably prudent الحكيمmember of the profession would use under similar circumstances, which leads to harm. (Applies ONLY to professionals such as licensed nurses).

Negligence and Malpractice

- Negligence and malpractice are the common terms for unintentional torts.
- Negligence thus denotes conduct that is lacking in care and is usually seen as carelessness on the part of the care provider.
- Malpractice, sometimes referred to as professional negligence, evolves from negligence law and the premise يفترض that all individuals are responsible for the consequences of their actions or inactions.

Elements of Malpractice

Elements of Malpractice and Examples of Actions That May Result in Malpractice		
Element	**Explanation**	**Examples of Actions**
Duty owed the person	Care should be given under the circumstances (what the prudent HCP would have done).	Failing to monitor a patient for postoperative bleeding
Breach of duty owed the patient	Not giving the care that should be given under the circumstances.	Failing to observe and report changes in a patient's condition.
Foreseeability of harm (proximate cause) an event sufficiently related to a legally recognizable injury	The nurse must have reasonable access to information about whether the possibility of harm exists.	Failing to act as a patient advocate, Example, the drug handbook specifies that the wrong route of dosage may cause injury. The nurse acts on behalf of the patient to clarify the correct route of administration.

Elements of Malpractice

Elements of Malpractice and Examples of Actions That May Result in Malpractice		
Element	**Explanation**	**Examples of Actions**
Causation (a direct relationship exists between failure to meet the standard of care and injury and can be proven)	Patient is harmed because proper care is not given.	Failing to question an inappropriate order
Injury	Actual harm results to patient.	Failing to provide patient education and discharge planning
Damages	Based on pain and suffering, expenses, emotional damages, or malicious damages.	Giving the patient incorrect medications

Cont. Divisions or Types of Law

2- Contract law "قانون العقود"

- Contracts are promises that the law will enforce.
- Contract law is that which governs the formation of these promises or agreements between two or more parties, in relation to a particular subject.
- Contracts can cover a broad range of matters, **including:**
  - the sale of goods or real property.
  - the terms of employment.
  - ownership of intellectual property developed as part of a work for hire.

Cont. Divisions or Types of Law

II- Criminal law "قانون الجناية"

- Criminal law, sometimes called public law, refers to laws that affect the state in its political capacity and that affect society as a whole.

Criminal law includes:

- **Misdemeanors** - Less severe crimes – جنحة

Ex: Petty Theft.

- **Felonies** - جريمة More serious than misdemeanors, and are usually punishable by more than one year in prison.

  Ex: Murder, Robbery.

Actions to Decrease the Risk of Liability

1. Communicate with your patients by keeping them informed and listening to what they say.
   Treat patients and their family with kindness and respect.
2. Acknowledge unfortunate incidents, and express concern about these events without either taking the blame, blaming others, or reacting defensively.
3. Chart and time your observations immediately, while facts are still fresh in your mind.
4. Take appropriate actions to meet the patient's needs.
   Be assertive and professional.
5. Complete incident reports immediately after incidents occur.

Actions to Decrease the Risk of Liability

6. Maintain clinical competency and professional certifications.
7. Acknowledge your limitations. If you do not know how to do something, ask for help.
8. Promptly report any concern regarding the quality of care (including the lack of resources with which to provide care) to your administration representative.
9. Delegate patient care based on the documented skills of licensed and unlicensed personnel.
10. Document the reason for any omission or deviation from agency policy, procedure, or standard.

Actions to Decrease the Risk of Liability

11. Follow evidence-based standards of care and the facility's policies and procedures for administering care and reporting incidents.
12. Avoid taking telephone and verbal orders. If needed, repeat the order back to the practitioner to assure clarity. Document that you did this with Verbal Order Repeated Back (VORB).
13. Encourage the development of clearly written and/or computerized orders from all practitioners.
14. Document the time of your actions and changes in conditions requiring notification of the practitioner. Include the response of the practitioner.

Reducing nursing liability

- **Liability area and Prevention measures**
  - Competent practice
  - Charting
  - Access to medical records (To whom Authorized)
  - Common sources of injuries
    - **Falls**
    - **Medications (Order and administration "6 Rights")**
    - **Restraints**
    - **Inadequate patient education**
    - **Abandonment (Not to arrange for continuing care)**
    - **Malfunctioning equipment**

Liability for Negligence by Students

- Nursing students have the ultimate responsibility for their own actions.

- Nursing students remain accountable for their knowledge and skills, and faculty or staff may become accountable for individual student incompetence or lack of preparedness.

Nurse practice acts

- **State statutes**

  Each state has statutes, or nurse practice acts, that define the levels of nursing, for example, advanced practice nurse [nurse practitioner, clinical nurse specialist, certified nurse midwife, certified nurse anesthetist].

Nurse practice acts

- **Disciplinary actions (اجراءات تأديبية)**
  - The state board of nursing may take action against a nurse who violates يخالف the nurse practice act or the licensing board's regulations.
  - A nurse can be disciplined for habitual substance abuse affecting the ability to practice, fraud or deceit in obtaining a nursing license, incompetence العجز , criminal felony conviction إدانة بارتكاب جريمة , and unprofessional conduct سلوك غير مهني

Nurse practice acts

- **Possible sanctions (عقوبات محتملة)**
  - The nurse's license may be revoked إلغاء
  - The license may be suspended for a predetermined time.
  - The license may be suspended for an undetermined time, and the nurse may reapply after completing course of study or treatment.
  - Conditions may be placed on the license to limit the nurse's practice.

Nurse practice acts

- **Possible sanctions (عقوبات محتملة)**
  - The nurse may voluntarily surrender the license تسليم الترخيص بشكل طوعي
    or enter into a consensual agreement with the nursing board, agreeing to limitations on the license or supervision of practice for a time.
  - The nurse may receive a disciplinary warning or reprimand توبيخ / تأنيب

Incident Reports

- Incident reports can't be used in court, unless they (lawyers) find out that it exists. They are intended for internal shit only, within the hospital, monitoring trends, prevention of future occurrences, etc.
- Don't put them in the chart! You can document the fall, what you did to make it better, but not that you filled out the actual incident report form.
- Don't document in pt's chart that you filled one out.

Incident Reports

- Don't tell the pt. or family that you're filling one out! You're not keeping them from any information about the incident, you're just not telling them about the form.
- Notify your manager when you fill it out.
- Remember, there is no law about having to fill out an incident report. It's just the hospitals policy to keep tabs on all the shit that goes wrong inside its walls.

Cont. Incident Reports

- Unusual / unexpected incidents

Do

  - Document incident information, treatment & follow up on chart
  - Notify your Manager

DO NOT

  - Leave copy on chart
  - Discuss with pt. / family
  - Document form completion in chart

Consent Form

Definition:

- Informed consent is a process of communication between a client and a member of the healthcare team that results in the client's authorization or agreement to undergo a specific intervention/procedure.
- It is more than simply getting a client to sign a consent form.
- Based on a full disclosure of risks, benefits, alternatives and consequences of refusal.
- Informed consent creates a legal duty for the physician and/or healthcare provider to disclose material facts in terms the client can reasonably understand to make an informed choice.

Cont. Consent Form

Principles:

In obtaining informed consent from the client, we need to consider the following:

1. Client receives sufficient information before making a decision for an informed consent.
2. Client is mentally competent when giving consent.
3. Client gives the consent voluntarily and is not under any pressure.
4. Consent is given to cover a specific intervention/procedure.

Cont. Consent Form

5. Client has the right to refuse or withdraw consent to treatment.
6. If it is in the best interest of the client or the public, medical examination/treatment can be done without consent under **the following situations:**

❖ Prevention of the spread of infectious diseases.

❖ Provision of emergency treatment under the common laws in unconscious adult client or in conscious client but incapable of giving consent.

Cont. Consent Form

7. Age limit is not the primary concern for a valid informed consent.

The general principle is that the client is able to understand fully the information given and its implications when he/she gives consent by himself/herself.

8. Proper documentation is an integral part of the whole consent process.

Cont. Consent Form

Following factors are required for informed consent:

1. A brief, complete explanation of the procedure or treatment must be given.
2. Names and qualifications of persons performing and assisting in the procedure should be provided.
3. A description of the serious harms, including death, which may occur as a result of the procedure, as well as anticipated pain and/or discomfort, should be provided.

Cont. Consent Form

4. An explanation of alternative therapies to the proposed procedure should be provided, as well as the risks of doing nothing.
5. The client must be advised on his or her right to refuse the procedure or treatment without discontinuing other supportive care.
6. The client may refuse the treatment even after it has begun.

Cont. Consent Form

Who May Give Informed Consent?

1. If the patient is competent, the patient has the right to consent to or refuse the treatment.

❖"Competency" is generally defined as the ability to understand the nature and consequences of the illness and the proposed treatment and alternative to treatment and the ability to make a reasoned decision in this regard.

❖The law presumes the competency of every person 18 years of age or older.

2. Parents are usually the legal guardians for pediatric patients and will sign the consent forms.

Cont. Consent Form

3. Sometimes, getting informed consent is difficult, for example, the client is unconscious. Then, consent can be obtained from a person legally authorized to give it on the patient's behalf, like a wife or husband.
4. In emergency situations, if it is impossible to obtain consent from the client or an authorized person, the procedure required to benefit or save the patient's life may be done without liability for failure to obtain consent. In such cases the law assumes the client would wish to be treated.
5. Psychiatric clients must also give consent. They retain the right to refuse treatment until a court has legally determined that they are incompetent to decide for themselves.

Cont. Consent Form

Revocation of Consent رفض الموافقة بعد التوقيع

- The health professional also has an important role if the patient subsequently wishes to revoke his or her prior consent or if it becomes obvious that the patient's already signed.
- In such a case, it is the health professional's responsibility to contact his immediate supervisor and the responsible physician. Both entities need to be informed of the patient's change of mind or lack of comprehension.

Ethical dilemmas

- **Ethical dilemmas by definition** involve two or more unfavorable alternatives to a given situation.
- Dilemmas that pose a choice between perplexing alternatives in the delivery of health care because of the lack of a clear sense of right or wrong.
- Nurses should consider the dilemmas that might arise in a given practice setting.
- **Example: "DNR"**

Cont. Ethical dilemmas

Types of dilemmas

- Dilemmas of fidelity- dilemmas that involve honoring promises.
- Dilemmas of nonmaleficence- dilemmas that involve the avoidance of harm.
- Dilemmas of confidentiality- dilemmas that involve respecting privileged information.
- Dilemmas of veracity- dilemmas that involve telling or concealing the truth.

Cont. Ethical dilemmas

One means of addressing such situations is to use ethical decision making models. Ethical decision making involves asking the following questions:

1. Who should make the choices?
2. What are the possible options or courses of action?
3. What are the available and reasonable alternatives?
4. What are the consequences, both positive and negative, of all possible options?
5. Which rules, obligations, and values should direct the choices?
6. What are the desired goals and outcomes in the given situation?

Cont. Ethical dilemmas

The easiest ethical decision making models to use in clinical settings is the MORAL model.

	Mean
M	Massage the dilemma.
O	Outline the options.
R	Resolve the dilemma.
A	Act by applying the chosen option.
L	Look back and evaluate.

Cont. Ethical dilemmas

Case Analysis Demonstration

Allison P. is a charge nurse on a busy medical surgical unit. She is expecting the clinical instructor from the local university at 2 p.m. to review and discuss potential patient assignments for the nursing students the following day. Just as the university instructor arrives, one of the patients on the unit develops a crisis requiring Allison's attention. In order to expedite the student nurse assignments for the following day, Allison gives her electronic medical record access password to the instructor.

Cont. Ethical dilemmas

Examine the ethical dilemma:

- Allison made a commitment to meet with the university instructor to develop student assignments at 2 p.m.
- The patient emergency that developed prevented Allison from living up to that commitment.
- Allison had an obligation to provide patient care during the emergency and a competing obligation to the instructor.
- She solved the dilemma of competing obligations by providing her electronic medical record access password to the university instructor.

Cont. Ethical dilemmas

Examine the ethical dilemma:

- By sharing her password, Allison most likely violated hospital policy related to the security of healthcare information.

Cont. Ethical dilemmas

Thoroughly comprehend the possible alternatives available:

1. Allison could have asked the instructor to wait until the patient crisis was solved.
2. Allison could have delegated another staff member to assist the university instructor.
3. Allison could have logged on to the system for the instructor.

Cont. Ethical dilemmas

Thinking in Consequences of alternatives available:

1. Allison could have asked the instructor to wait until the patient crisis was solved.

Good Consequences

- No policy violation
- Patient right safeguarded

Bad Consequences

- Not the best use of the instructor's time

Cont. Ethical dilemmas

Thinking in Consequences of alternatives available:

2. Allison could have delegated another staff member to assist the university instructor.

Good Consequences

- No policy violation

Bad Consequences

- Other staff may be equally busy
- May not be as familiar with all patients

Cont. Ethical dilemmas

Thinking in Consequences of alternatives available:

3. Allison could have logged on to the system for the instructor.

Good Consequences

- Instructor can begin making assignments.

Bad Consequences

- May still be a violation of policy regarding system access.

From the analysis, it is clear that the best immediate solution is to delegate assisting the instructor with assignments to another nurse on the unit.

Ethical decision making

What is Ethical Decision Making?

- When faced with an ethical dilemma the objective is to make a judgment based on well-reasoned, defensible ethical principles.
- The risk is poor judgment i.e. a low-quality decision.
- A low-quality decision can have a wide range of negative consequences.

Cont. Ethical decision making

Types of decisions:

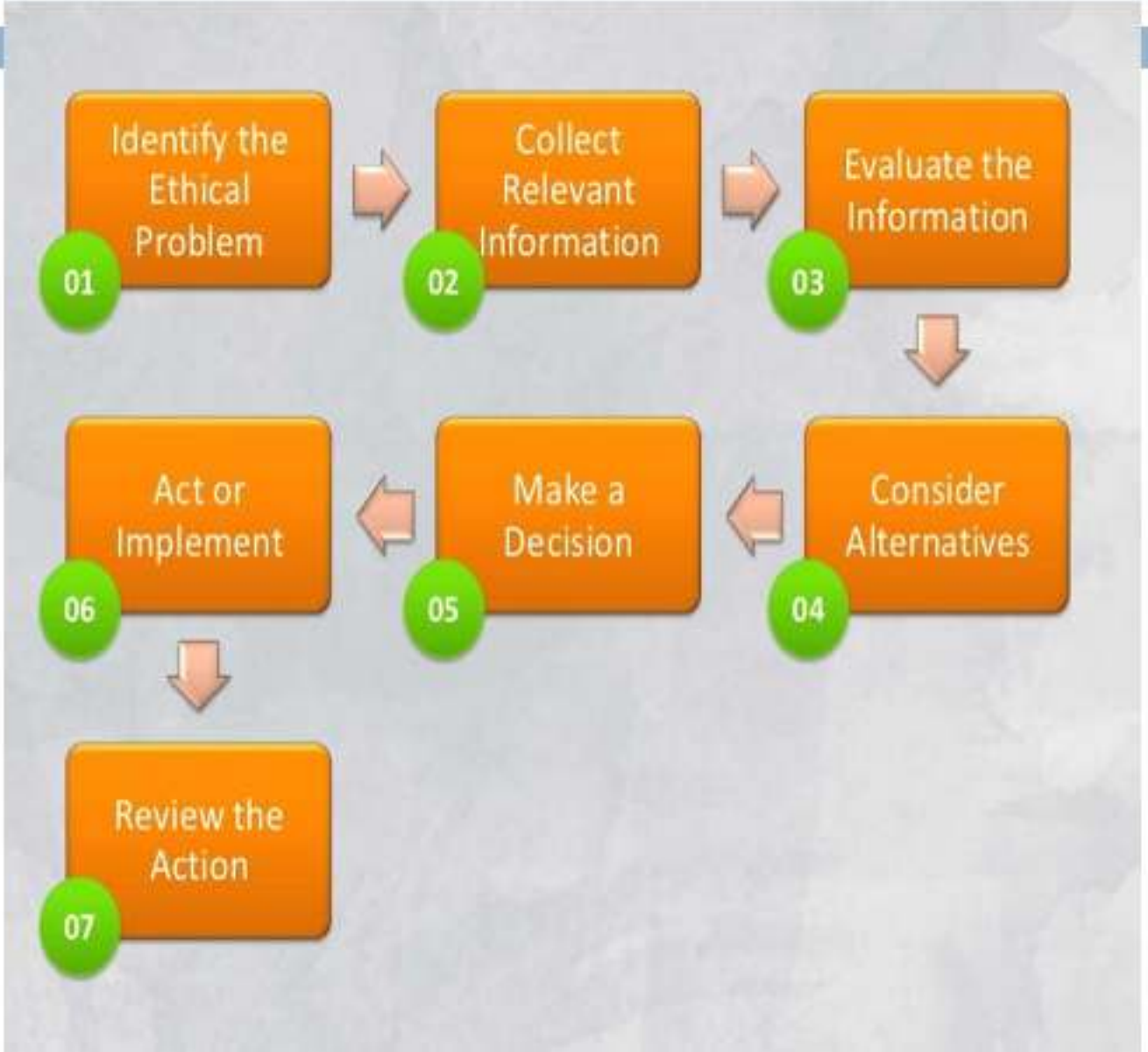
- **Active decisions-** ethical decisions and moral judgments that lead directly to actions and bring about changes.
- **Passive decisions-** decisions that deny, delay, or avoid action and maintain the "status-quo" الوضع الراهنby denying or shifting responsibility to avoid changes.
- **Programmed decisions-** decisions that use precedents, established guidelines, procedures, and rules to resolve anticipated متوقع routine, expected types of moral dilemmas.
- **Non-programmed decisions-** decisions that require a unique response to complex and unexpected moral dilemmas.

Cont. Ethical decision making

A framework for Ethical Decision Making (EDM):

- Identify and clarify the ethical problem
- Gather data
- Identify options
- Make a decision
- Act and assess

A framework for Ethical Decision Making (EDM)



CLINICAL APPLICATION

- You are the night shift nurse caring for Mr. Ali. In the morning, Mr. Ali is scheduled for a permanent pacemaker insertion due to third degree heart block.
- Hospital policy state that no visitor may stay all night with a patient unless that patient is very critically ill. Mr. and Mrs. Ali are both requesting that Mrs. Ali stay in a chair beside Mr. Ali's bed, because both are anxious about his upcoming procedure.
- Use your decision-making skills to help you decide what to do.

CLINICAL APPLICATION

Step 1: Identify the need for a decision

- You let Mrs. Ali spend the night? Consider all the information (hospital policy, the patient's wishes, anxiety level, and so on).

Step 2: Determine the goal or outcome

Questions to consider include the following:

- Can an exception be made to hospital policy?
- Is the goal to alleviate Mr. and Mrs. Ali's anxiety?
- Will Mr. Ali's level of anxiety affect the outcome of the surgery?
- Will Mr. and Mrs. Ali be satisfied customers?

CLINICAL APPLICATION

Step 3: Identify many alternatives

- i.e., personal, financial, political, etc., and the benefits and consequences of each.
- If you enforce hospital policy, the benefits are that all patients are treated equally and the written policy supports the decision.
- The other alternative is to allow Mrs. Ali to stay all night, the benefits are that Mr. and Mrs. Ali's level of anxiety will decrease and they will be satisfied customers.

CLINICAL APPLICATION

Step 4: Make the decision

- Consider all alternatives and the benefits and consequences of each, and then implement the decision.
- The decision was made to let Mrs. Ali spend the night with her husband.
- The nurse determined that the couple's anxiety level in relation to the outcome of the surgery was the most important consideration and the goal in this scenario.

CLINICAL APPLICATION

Step 5: Evaluate the decision

- Was the goal achieved?
- The nurse must evaluate the decision that was implemented.
- The decision to allow Mrs. Ali to spend the night was a good decision.
- It allowed Mr. Ali to get a good night's rest and also decreased his overall anxiety.
- Decreasing a patient's anxiety prior to a surgery is an important medical intervention.
- This decision might lead to a change in the current policy, allowing for more customer satisfaction.



Box 42–5 **Examples of Nurses' Obligations in Ethical Decisions**

- Maximize the client's well-being.
- Balance the client's need for autonomy with family members' responsibilities for the client's well-being.
- Support each family member and enhance the family support system.
- Carry out hospital policies.
- Protect other clients' well-being.
- Protect the nurse's own standards of care.

Ethical committees

- Ethical committees or select members often help resolve ethical conflicts and answer ethical questions through the provision of consultations. (تقديم الاستشارات)
- Ethics committees can lead to the establishment of policies and procedures for the prevention and resolution of dilemmas.
- Ethical decisions are complex, especially when they affect the live of clients.
- Ethical dilemmas span the age continuum from pre-birth to death and post death.



Cont. Ethical committees

- Because making a judgment for another is a weighty responsibility, many health agencies have established ethical committees.
- It is comprised of an interdisciplinary group representing medicine, nursing, religious care, pharmacy, nutritional services, social services quality managements, legal services and community.
- They are also called on to offer advice, to protect clients best interest and to avoid legal battles.

Cont. Ethical committees

The mission of an ethical committee:

1. Provide thoughtful and timely consultation when an ethical issue arises.
2. Involve an emergency meeting of some or all the members.
3. Put options along with the risk and benefits of options.
4. Examine the resolutions and potential outcomes of those resolutions.
5. The committee consults, give guidance and provide any resources needed.
6. The committee does not make decisions.

Ethical Issues

What is an 'ethical issue'?

- When you have to judge what is right or wrong.
- Choosing between options.
- Deciding whether to do something or do nothing.
- Should I or shouldn't I?
- Weighing up the potential impact of your decisions or actions.

Cont. Ethical Issues

I- Euthanasia القتل الرحيم – موت الرحمة

- Euthanasia; meaning "**good death**"

eu (well or good) + thanatos (death)

Refers to the practice of intentionally ending a life in order to relieve pain and suffering.

Forms of Euthanasia

1- Active *(Euthanasia by action)* – death is brought by an ACT.

It entails the use of lethal substances or forces to kill.
This is the most controversial form of euthanasia.

2- Passive *(Euthanasia by omission)* – death is brought about by an OMISSION.

Intentionally causing death by not providing necessary and ordinary (usual and customary) care or food and water.
It entails the withholding of treatments such as administration of antibiotics or any drugs or intervention necessary for the continuance of life. Switching off the machine that keeps the person alive and withholding the surgery that may extend the life of the person is under this form of euthanasia.

Classification of Euthanasia

1- Voluntary euthanasia: When the person who is killed has requested to be killed, this type of euthanasia is often termed as assisted suicide; As someone provides an individual with the information, guidance, and means to take his or her own life with the intention that they will be used for this purpose. When it is a doctor who helps another person to kill themselves it is called "physician assisted suicide."

Voluntary euthanasia is made legal in countries such as Belgium, the Netherlands, Switzerland, Luxemburg, and the US states of Oregon and Washington.

- **Euthanasia is legal in Belgium, the Netherlands , Canada, and Luxembourg. Assisted suicide, where the patient has to take the final action themselves (unlike euthanasia), is legal in the Netherlands, Luxembourg and Switzerland. In the United States there are assisted dying laws restricted to terminally ill and mentally competent adults in Oregon, Montana, Washington, New Jersey[12] and Vermont.**

Cont. Classification of Euthanasia

2- Non-voluntary: When the person who is killed made no request and gave no consent,

e.g. when a client is unconscious or is still a child and unable to make meaningful choices between living and dying, and an appropriate person takes the decision on their behalf non-voluntary euthanasia is practiced.

Non-voluntary euthanasia is illegal in all countries.

Cont. Classification of Euthanasia

3- Involuntary euthanasia: When the person who is killed made an expressed wish to the contrary (in cases where the dying client chooses life but is killed anyway).

Involuntary euthanasia is usually considered murder.

Ethical Issues

II- Organ donation التبرع بالاعضاء

- Organ donation is the donation of biological tissue or an organ of the human body, from a living or dead person to a living recipient in need of a transplantation. Transplantable organs and tissues are removed in a surgical procedure following a determination, based on the donor's medical and social history, of which are suitable for transplantation.

Cont. Organ donation

- Organ donation is conditional in Islam

- ترجيح القول بجواز التبرع بالأعضاء، إذا لم يكن التبرع بها يؤدي إلى وفاة صاحبها .

Cont. Ethical Issues

III- In Vitro Fertilization الاخصاب المخبري – أطفال الانابيب

- In vitro fertilization (IVF) is a process by which an egg is fertilized by a sperm outside the body (in vitro). IVF is a major treatment for infertility when other methods of assisted reproductive technology have failed.

- In vitro fertilization is conditional in Islam

Cont. Ethical Issues

IV- Human cloning الاستنساخ البشري

- Human cloning is the creation of a genetically identical copy of a human.
- Human cloning is forbidden in Islam

الاستنساخ طريقة شاذة في تكاثر البشر، وخروج سافر على ناموس الله في الكون وتغيير ظاهر لخلق الله عز وجل، قال تعالى: (وَلَآمُرَنَّهُمْ فَلَيُغَيِّرُنَّ خَلْقَ اللَّهِ) [النساء:١١٩]. وفيه تعرض الأجنة المستنسخة للبيع والتشويه، فإن الاستنساخ يفضي إلى وجود أجنة فائضة ليس أمامها إلا الموت أو الاستزراع في أرحام نساء أخريات، فإن تركت للموت كان مؤدى هذه الطريقة هو التسبب في إنشاء حياة ثم تركها تموت، وإن أودعت في أرحام أخريات غير الأم كان مؤدى ذلك أن تحمل هذه الأرحام أجنة غريبة عنها، وكل ذلك لايجوز شرعاً وخُلُقاً وعقلاً.

Cont. Ethical Issues

V- Surrogate Mother (الرحم المستأجرة)

- Surrogacy is an arrangement in which a woman carries and delivers a child for another couple or person.

- The surrogate may be the child's genetic mother (called traditional surrogacy), or she may be genetically unrelated to the child (called gestational surrogacy).

Cont. Surrogate Mother

- In a **traditional surrogacy**, the child may be conceived via artificial insemination using fresh or frozen sperm or impregnated via IUI (intrauterine insemination), or ICI (intracervical insemination) performed at a health clinic.
- A **gestational surrogacy** requires the transfer of a previously created embryo, and for this reason the process always takes place in a clinical setting.

Cont. Surrogate Mother

- **The traditional surrogacy and gestational surrogacy are forbidden in Islam.**

- صدر قرار من مجمع الفقه الإسلامي التابع لرابطة العالم الإسلامي في دورته الخامسة سنة ١٤٠٢هـ بتحريم هذا الأسلوب من أساليب التلقيح .
  كما صدر قرار من مجمع الفقه الإسلامي التابع لمنظمة المؤتمر الإسلامي في دورة مؤتمره الثالث سنة ١٤٠٧هـ بتحريمه أيضاً.

Cont. Ethical Issues

VI- Family Planning

Family planning is the planning of when to have children, and the use of family planning methods to implement such plans.

- Family planning is conditional in Islam

Cont. Family Planning

تحديد النسل خوفاً من ضيق الرزق لا يجوز، لأن الرزق بيد الله عز وجل، فهو الذي قدر الآجال والأرزاق وما من مولود يولد إلا وقد قدر له رزقه كما قدر له أجله، والله سبحانه وتعالى يقول: {وَلاَ تَقْتُلُواْ أَوْلاَدَكُم مِّنْ إمْلاَقٍ نَّحْنُ نَرْزُقُكُمْ وَإِيَّاهُمْ} [سورة الأنعام: آية ١٥١]، فهذا فيه شبه من فعل الجاهلية الذين يقتلون أولادهم خشية الفقر، إلا أن هذا يعتبر منعاً لحصول الأولاد خشية الفقر والجاهلية يقتلون الأولاد الموجودين خشية الفقر.

Cont. Family Planning

وعلى كلٍّ فالعلة واحدة ولا يجوز مثل هذا، والأرزاق بيد الله عز وجل، وتحديد النسل خوفاً من الفقر فيه إساءة ظن بالله عز وجل، فعليك أن تتوكل على الله عز وجل، والله سبحانه وتعالى يرزق من يشاء بغير حساب، فأحسن الظن بربك ولا تتطرق إليك هذه الهواجس، فأنت لا تدري الخير والمصلحة، يقول الله سبحانه وتعالى: {وَعَسَى أَن تَكْرَهُواْ شَيْئاً وَهُوَ خَيْرٌ لَّكُمْ وَعَسَى أَن تُحِبُّواْ شَيْئاً وَهُوَ شَرٌّ لَّكُمْ وَاللّهُ يَعْلَمُ وَأَنتُمْ لاَ تَعْلَمُونَ} [سورة البقرة: آية٢١٦].

Cont. Family Planning

وإذا كان هذا التنظيم أو تأخير الحمل لداعٍ صحي بالمرأة ككون المرأة مثلاً لا تطيق الحمل والولادة في حالة خاصة، أو ظرف خاص لمرضها فإنه لا مانع من أن تعطي ما يمنع الحمل مؤقتاً حتى تزول هذه الحالة التي يشق عليها فيها الحمل والولادة، فهذا يكون من باب الوقاية والعلاج لا من باب تحديد النسل خشية الفقر.

Death And End Of Life Ethics

- Death remains a great mystery, one of the central issues with which religion and philosophy and science have wrestled since the beginning of human history. Even though dying is a natural part of existence.

- إن الله تعالى خلق الموت والحياة، كما قال سبحانه وتعالى: (الَّذِي خَلَقَ الْمَوْتَ وَالْحَيَاةَ لِيَبْلُوَكُمْ أَيُّكُمْ أَحْسَنُ عَمَلاً وَهُوَ الْعَزِيزُ الْغَفُورُ) [الملك:٢] .

Death

- قال الله تعالى في سورة الإسراء: {وَيَسْأَلُونَكَ عَنِ الرُّوحِ قُلِ الرُّوحُ مِنْ أَمْرِ رَبِّي وَمَا أُوتِيتُم مِّن الْعِلْمِ إِلاَّ قَلِيلاً}

- لم يجب الله عن «الروح»، بل بيَّن أنها من خصوصياته سبحانه وتعالى، وأنه هو الذي خلقها، وهو الذي يعلمها ولا يعلمها أحد من الخلق، فهي سرٌّ من الأسرار، ولا تزال سرًّا، وهذا من معجزات القرآن، فإنه مع تقدم الطب والمهارة فيه ومع حرص الناس على البحث في هذا الشأن لم يعرفوا شيئًا عن حقيقة الروح.

Loss

- Birth, loss, and death are universal and individually unique events of human experience. Life is a series of losses and gains.

- ***Loss:*** A person experiences loss in the absence of an object, person, body part or function of emotion that was formerly present.

Loss

Categories of Loss:

- Loss of external objects e.g. possession that is worn out, misplaced, stolen or ruined by disaster.
- Loss of known environment e.g. separation from a known environment includes leaving a familiar setting.
- Loss of a significant others e.g. include parents, spouses, children, siblings ...etc.
- Loss of an aspect of self e.g. body part, physiological function, psychological function.... etc..
- Loss of life: persons who face death live, feel, think and respond to events and people around them until the moment of death.

Signs of Close Death

- Lose ability to perform ADLs without assistance.
- Lose interest in food and drink.
- Increased sleep.
- Begin to display altered mental status (delirium, confusion, lethargy).

Imminent Death الموت الوشيك

- Increased cognitive impairment.
- Unstable vital signs.
- No food or drink.
- Decreased urine/stool output.
- Pulmonary Congestion/increased secretions.
- Mottling/pallor/cyanosis.
- Edema/anasarca.
- Altered breathing.
- Cool extremities.
- Limited or no blinking.

Table 10–12 Physiologic and Psychosocial Symptoms of Dying

Physiologic Symptoms	Psychosocial Symptoms
Inability to swallow	Denial and isolation
Decreased gastrointestinal and urinary tract activity	Anger
Bowel and bladder incontinence	Bargaining
Loss of motion, sensation, reflexes	Depression
Elevated temperature coupled with cold, clammy skin	Acceptance
Cyanosis	
Lowered blood pressure	
Irregular respirations; Cheyne–Stokes respirations	

Responses to death and dying

- **Elisabeth Kübler-Ross** discovered that most people pass through five stages in response to death and dying.
- Although some individuals experience the stages in a different order and others revisit some stages, knowledge of these stages can help the H.C.P. evaluate and deal with a patient's responses to death and dying.

Cont. Responses to death and dying

1. **Denial and isolation**: the initial response characterized by shock and disbelief.
2. **Anger**: expressions of rage and resentment.
3. **Bargaining**: attempts to strike a bargain, typically with God, in exchange for prolonged life.
4. **Depression**: feelings of loss, grief, and intense sadness.
5. **Acceptance**: a quiet stage characterized by a gradual, peaceful withdrawal from life.

Health Care Implications

For a dying patient's psychosocial and spiritual needs:

- Help the patient make final plans.
- Help the patient control personal routines and affairs as long as possible.
- Provide opportunities for reminiscence and validation of self worth احترام الذات
- Support the patient's expression of difficult feelings.
- Use touch as a communication tool, if acceptable to the patient.
- Provide for formal spiritual support, such as religious visits or last rites.

Cont. Health Care Implications

For a dying patient's physical needs:

- Regularly perform routine personal care measures, such as turning, positioning, bathing, and grooming.
- Be alert for constipation, a common problem.
- Anticipate bowel and bladder incontinence as death nears.
- Be sensitive to individual preferences for lighting and noise control.
- Use pain medications and comfort measures as needed to maintain patient comfort.
- Offer favorite liquids and soft foods; appetite wanes as death nears.
- Remember that hearing is the last sense to be lost.

Cont. Health Care Implications

For family members' psychosocial and spiritual needs:

- Allow flexible visitations.
- Encourage family members to participate in patient care.
- Recommend rest breaks for family members.
- Visit the patient regularly, and perform routine personal care activities while family members are present.
- Encourage the family to personalize the patient's room.
- Provide for bereavement follow-up.
- Recognize the need for formal counseling in difficult situations.
- Encourage the use of support groups when available.

The dying person's bill of rights

- Have the right to be treated as a living human being until death.
- Have the right to maintain a sense of hopefulness.
- Have the right to participate in decision concerning his care.
- Have the right to expect continuing medical and nursing attention even though: “cure” goals must be changed to "comfort" goal.
- Have the right not to die alone.

Cont. The dying person's bill of rights

- Have the right to be free from pain.
- Have the right to have questions answered honestly & not to be deceived.
- Have the right to die in peace and dignity.
- Have a right to retain individuality and not be judged for his decisions, which may be contrary to beliefs of others.
- Have the right to discuss and enlarge his religious and / or spiritual experiences.

Cont. The dying person's bill of rights

- Have the right to expect that the sanctity of the human body will be respected after death.
- Have the right to be cared for by caring, sensitive, knowledgeable people who will attempt to understand his needs and will be able to gain some satisfaction in helping him.

Table 10–13 Ethical and Legal Dimensions

Advance Directives	Living will: provides specific instructions about the kind of health care that should be provided or foregone in particular situations. Durable power of attorney for health care: appoints an agent the client trusts to make decisions in the event of incapacity.
Do-Not-Resuscitate (DNR)/No-Code orders	No attempts are to be made to resuscitate a client who stops breathing or whose heart stops beating.
Comfort-Measures-Only order Do-Not-Hospitalize order	The goal of treatment is comfortable, dignified death without implementation of life-sustaining measures. Utilized by clients living in long-term care and assisted living facilities who do not wish to be hospitalized for aggressive treatments.

Table 10–14 Nursing Interventions at End of Life

Communicate openly with the client and family regarding the dying process.	Provide necessary information for the client and family to engage in the decision-making process regarding end-of-life care.
Be cognizant of death-related beliefs and practices of clients of other cultures.	Facilitate spiritual care at the client's and family's request.
Manage client pain and dyspnea with pharmacological and nonpharmacological interventions.	Provide nutrition and hydration as indicated or when appropriate.
Provide a safe environment for clients who are weak, confused, or delirious.	Provide pharmacological and nonpharmacological interventions for the management of depression.
Identify situational role changes and provide support.	Provide after-death care and bereavement support to the family.

Nurses' pledge

- I solemnly pledge myself to the service of humanity and will endeavour to practice my profession with conscience and with dignity.
- I will maintain by all the means in my power the honour and the noble traditions of my profession. The total health of my patients will be my first consideration.
- I will hold in confidence all personal matters coming to my knowledge.
- I will not permit considerations of religion, nationality, race or social standing to intervene between my duty and my patient.
- I will maintain the utmost respect for human life.
- I make these promises solemnly, freely and upon my honour.

قسم التمريض

أُقسم بالله العظيم أن أكون مخلصًا في عملي ، رحيمًا بمرضاي مراعيًا لمشاعرهم ، مدافعاً عن حقوقهم ، أمينًا علي أسرارهم ، محترمًا لمعتقداتهم ومحافظًا علي خصوصياتهم.

كما و أعاهد الله علي ألا أتعاطى أو أعطي ما يتعارض مع الدين الإسلامي وآداب مهنة التمريض وأخلاقها، وأن أحافظ علي صحة وسلامة الإنسان.

وأن أكون متعاونًا مع زملاء المهنة والمهن الأخرى من أجل خدمة ومصلحة المرضي والإنسانية ما استطعت إلي ذلك سبيلا..

والله علي ما أقول شهيد،،،